

CDPH CAB Online Project

What This Procurement Is About — A Plain-Language Analysis

Solicitation: RFP 26-10053 A5 (CDT STP Request for Proposal)

Customer: California Department of Public Health (CDPH)

Conducted by: California Department of Technology (CDT), Office of Statewide Technology Procurement

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Based on: RFP Part 1 (Bidder Instructions), Exhibit A (Statement of Work),
Exhibit B (Requirements), Exhibit C (Cost Worksheets), Exhibit D (Deliverables Table),
and public CDPH sources

1. Executive Summary — the Project in Plain Terms

The California Department of Public Health (CDPH) licenses and certifies more than **14,000 healthcare facilities** in California — hospitals, clinics, skilled nursing facilities, hospices, home health agencies, and about 30 other provider types. The unit that processes all of those license applications is the **Centralized Applications Branch (CAB)**, part of the Center for Health Care Quality (CHCQ), Licensing & Certification Division.

Today, out of 35 provider types, **only 2 — General Acute Care Hospitals (GACH) and Acute Psychiatric Hospitals (APH) — can apply online**. Everyone else mails a paper application packet with a paper check. CDPH staff then manually re-key that data into the department's licensing system of record, the Electronic Licensing Management System (ELMS), and manually verify applicant information against roughly a dozen external databases. The result: 72% of applications arrive incomplete or incorrect, the average open Initial/Change-of-Ownership application is **231 days old**, and staff track statutory deadlines with spreadsheets and email.

The CAB Online project buys a contractor to design, develop, and implement (DD&I) an expansion of the existing online application system so all remaining ~33 facility types can apply, pay, and track their applications electronically — and so the data flows automatically into ELMS and the verification checks run automatically against external systems. It is an *enhancement and extension of an existing application*, not a green-field build: the existing GACH-APH online submission system (originally built on Adobe Experience Manager in December 2019, and being migrated by CDPH to Microsoft Power Platform in early 2026, before this contract starts) is the foundation that CAB Online will be built on top of.

One-sentence version: Take the online license application portal that today serves only 2 hospital types, rebuild-and-extend it on Microsoft Power Platform / Dynamics 365 / Azure inside CDPH's own cloud tenants, roll it out to all ~33 remaining healthcare facility types in three phases, connect it to ELMS and a dozen verification systems, add electronic payments — and then run, support, and maintain it.

Contract term	Platform	Delivery model	Cost weight	Availability SLA
38 months base (26 dev + 12 M&O), up to 86 with options	MS Dynamics 365, Power Platform, Azure (CDPH tenants)	3 phased go-lives, Agile Scrum, fixed-price deliverables	50% of evaluation points are cost	99.9% 24×7 production uptime

2. Who Is Buying and Why

CDPH sits within the California Health and Human Services Agency (CalHHS). Its Center for Health Care Quality (CHCQ) operates the Licensing and Certification (L&C) Division, which protects patient safety across California's

healthcare facilities. CHCQ is funded primarily by a CMS (Centers for Medicare & Medicaid Services) grant and by licensing fees. Within L&C, the **Centralized Applications Branch (CAB)** receives, reviews, and adjudicates:

- **Initial applications** — a new facility seeking a license (≈500–1,600/year);
- **Change of Ownership (CHOW)** applications (≈300–420/year);
- **Report of Change** applications — changes to beds, services, management, etc. (≈10,000–13,000/year);
- **License renewals** (≈13,000–15,900 facilities/year) and **certification workloads** (Medi-Cal enrollment, re-certification, adverse action requests, ≈3,300–4,300/year).

The procurement is run by the California Department of Technology (CDT) on CDPH's behalf under Public Contract Code §6611 (competitive negotiation). The contract is **fixed-price, deliverables-based**, awarded to a single bidder, with 15% of each payment withheld until Final System Acceptance.

The project is also strategic: it is a foundational piece of CDPH's "**Future of Public Health**" (**FoPH**) **Initiative 2**, which aims to consolidate transactional systems into one-stop, enterprise-scale platforms for licensing and surveillance. CAB Online is explicitly described as "laying the groundwork for the CDPH licensing landscape."

3. The Existing Applications — What Is In Place Today

Understanding the current environment is essential because the RFP is an **enhancement** of this environment. Three systems matter most, plus a set of look-up systems used for verification.

3.1 GACH-APH Online Application System (the system being extended)

In **December 2019**, CDPH implemented an online application submission system pursuant to Health & Safety Code §1272, using **Adobe Experience Manager (AEM)** — Adobe's enterprise web content / forms platform. It is exposed publicly through the CDPH "Certificates, Licenses, Permits and Registrations" portal (eforms.cdph.ca.gov). Since July 2020, paper applications for GACH and APH are no longer accepted.

What it does today:

- Lets GACH and APH providers create an account, and submit **Initial, Change of Ownership, and Report of Change** applications with supporting documents through guided web forms;
- Issues an Online Application ID (tracking number) by email on submission;
- Lets providers upload missing/corrected documents when CAB requests them;
- **Does not take payments** — even online applicants must still mail a business check, cashier's check, or money order;
- **Does not talk to ELMS** — CAB analysts manually re-key everything the provider typed into ELMS.

Critical timing fact: The GACH-APH system is being migrated by CDPH from AEM to **Microsoft Power Platform** under the FoPH initiative. That migration completes in Q1/Q2 2026, *before this contract executes*. The migrated Power Platform GACH-APH system is the platform baseline that the CAB Online contractor will extend. Bidders are therefore not migrating AEM — they are building on a fresh Power Platform implementation and expanding it to 33 more facility types while enhancing its functionality (payments, integrations, workflow, status tracking).

3.2 ELMS — Electronic Licensing Management System (the system of record)

ELMS is a CDPH-internal **web-forms application created in 2005**. It is the authoritative **licensing system of record**: it captures facility applications, issues licenses, generates license renewal notices and invoices, determines license/application fees, issues and tracks State enforcement actions, and generates management reports. ELMS is *not* being replaced by this project — CAB Online must **integrate bidirectionally with ELMS** so approved application data posts automatically without re-keying, and so CAB Online can pull existing licensing data (e.g., the facilities affiliated with an owner) in real time.

3.3 Supporting and verification systems

System	What it is / role today
ASPEN / ACO	Automated Survey Processing Environment (Central Office) — federal/state suite for survey & certification data; analysts look up provider compliance data here manually.
iQIES	CMS's Internet Quality Improvement and Evaluation System — web system tracking provider/facility compliance, survey and certification data (e.g., CMS-1539 forms).
CABS / OASYS	Caregiver Applicant Background System / its Online Application System — criminal background check processing for caregiver applicants (CNAs, HHAs).
RSS	Receiving, Staging, and Storing system — document intake staging.
First Data	The State's electronic payment processing system (payment portal) — CAB Online must route payments through it and feed transactions to ELMS.
Verification look-ups	Secretary of State (SOS) business search, DHCS PAVE (Medi-Cal enrollment) & PED, DCA license search (MD/RN licenses), OIG LEIE & SAM exclusion lists, IRS tax-exempt search, NPPES (NPI validation), NHA license registry, DOJ records, BreZEze, Accuzip (zip-code verification). Today all these checks are done manually by analysts, and results logged manually.

3.4 How the process works today (current state)

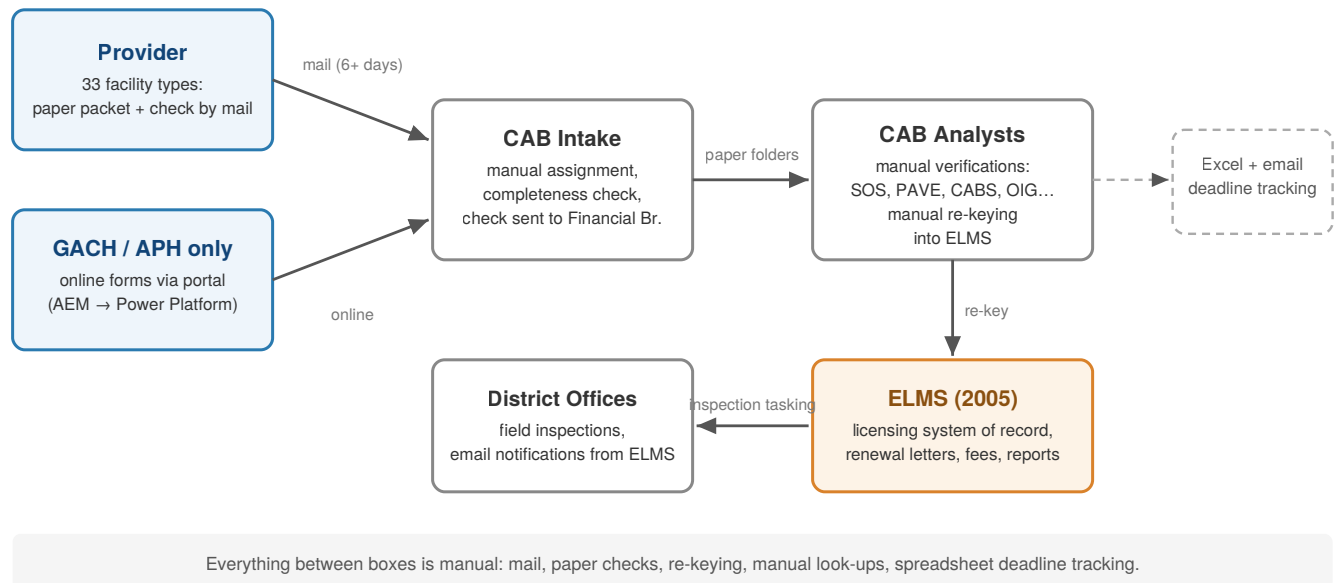


Figure 1 — Current-state application processing at CAB

3.5 The pain points the RFP itself lists

- Communicating by regular mail — every request takes at least six business days;
- Paper checks: bounced/underpaid checks, untraceable payments, refund complications, licensing delays;
- Sensitive information exchanged over non-secure channels (email);
- Duplicate data entry for programs that already receive applications through the online system;
- External tools (Excel) needed to track process deadlines mandated by law;
- Inability to detect license-related changes (relies on self-reporting);
- Difficulty confirming providers received renewal letters;
- No license-state tracking tools despite the data existing in the system.

The numbers behind the problem (ELMS records, 11/2020–11/2021): 86% of Initial, 78% of CHOW, and 69% of Report-of-Change applications were incomplete or needed correction — 8,366 of 11,574 applications (72%) triggered the 60-day correction cycle. The average open Initial/CHOW application is 231 days old, against statutory review timeframes of 30–120 days (HSC §1218, §1272, Title 22, AB 1502). The stated goal: cut time spent in correction status by **50%**.

4. What Will Be Built — the CAB Online Solution

4.1 Concept

CAB Online will be a **cloud-based SaaS solution built with Microsoft Dynamics 365, Power Platform, and Azure, inside CDPH's existing tenants** (an explicit, non-negotiable platform requirement — Section 2.2.5 of Part 1). The State owns the cloud infrastructure and Microsoft licenses (per-seat, in the State's name); the contractor configures, customizes, integrates, tests, trains, and operates. It will serve both internal CDPH users (~100 CAB staff, 16 district office staff, ~20 Fiscal staff) and external stakeholders (~14,000–15,000 provider facilities).

The system streamlines four core business processes: **(1) Application Intake, (2) Change Applications, (3) License Renewal, (4) License Revocation** (self-requested suspensions/terminations and non-payment revocations). In the future state:

- Providers complete applications in CAB Online, upload all supporting documents, and **pay electronically via the Payment Portal (First Data)**;
- Applications land in an **Application Pool** where intake analysts pre-screen; validation rules prevent incomplete submissions (CAB will stop accepting incomplete packages as a business-process change);
- **Verifications run automatically** — SOS, PAVE, restricted-provider lists, CABS, and others queried electronically with results logged in CAB Online;
- Approved data **posts automatically to ELMS** — no re-keying; ELMS remains the system of record and reporting source;
- Workflow shuttles applications electronically between CAB, District Offices, and the Fiscal Management Branch; field inspections are triggered and results logged;
- Providers see **application status online**, receive notifications, and download their license from the portal;
- Documents are stored in a document management system rather than shared network drives.

4.2 Future-state architecture

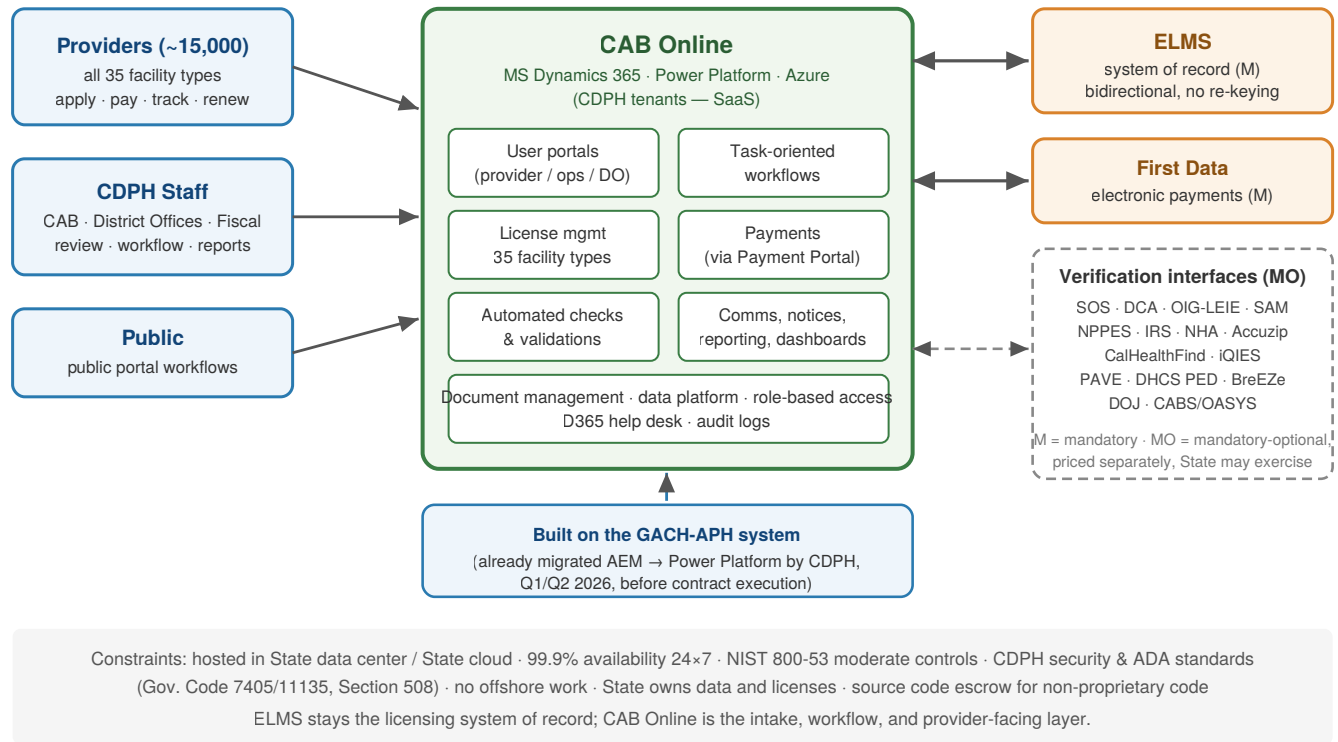


Figure 2 — Proposed CAB Online solution and its integration landscape

4.3 Scope of requirements (Exhibit B)

Exhibit B contains **729 functional requirements** (704 Mandatory, 24 Mandatory-Optional, 1 Desirable) and **407 non-functional / project-transition requirements**. The functional requirements are grouped into eight main capabilities:

Capability	~ Count	What it covers (examples)
Facility Licensing	151	Apply for each of the 30+ license/certification types; license management lifecycle; ELMS/PAVE/SOS data integration; status dashboards consistent with ELMS.
Portal	168	Provider, License-Operations (CAB analyst), District Office, Admin, Helpdesk, and Public portal workflows; user portals with role-based access.
Workflows	125	Task-oriented architecture: task libraries, task packages, workbench for building/ deploying workflows; provider and license-operations workflow automation.
Financial Management	73	Electronic payment via CDPH Payment Portal, invoices, refunds, payment plans, renewal fees, reconciliation reporting.
Communications	101	Provider notifications, secure messaging, templates and letters, helpdesk workflows, reporting.
Qualifications Management	32	Verify people and entities: credential stores, personnel qualifications, background-check integration (CABS/OASYS), professional license checks.

Data Platform	33	Structured/non-structured data management, registries, data integration pipelines, real-time ELMS queries.
Document Security	23	Secure document handling, storage, role-based document access.

Non-functional categories: Availability & Reliability (66), Installation/Project Transition (56), Extensibility & Configurability (52), Financial Operations/FinOps (49), Usability (45), Maintainability (39), QA & Testing (35), Interoperability (31), Performance & Capacity (31). Notable items: 99.9% uptime with fault tolerance and rapid capacity scaling, cross-platform and mobile-adaptive UI, internationalization support, and a ~20% overhead assumption to address backlog remaining in the GACH-APH/ELMS systems.

4.4 Interfaces (Exhibit A §24)

#	Interface	Direction	M/ MO	Purpose
1	ELMS	In/Out	M	System of record — ingest application data from CAB Online, send licensing data back; ingest First Data transactions.
2	First Data	In/Out	M	Electronic payment processing.
3	Accuzip	In	MO	Zip-code verification already built into Power Apps under GACH-APH migration.
4	DCA License Search	In/Out	MO	Auto-verify professional licenses (MD, RN...), save results (PDF) to ELMS.
5	OAG / OIG LEIE	In/Out	MO	Auto-check ≥5% owners against Medicare/Medi-Cal exclusion lists.
6	CalHealthFind	Out	MO	Send data to the public facility-lookup portal.
7	NPPES	In/Out	MO	Auto-validate NPI numbers.
8	Secretary of State	In/Out	MO	Auto-verify legal business name, entity standing.
9	SAM	In/Out	MO	Auto-check ≥5% owners against federal exclusions.
10	NHA License	In/Out	MO	Auto-verify nursing home administrator licenses for SNFs.
11	IRS Tax-Exempt Search	In/Out	MO	Verify nonprofits without SOS registration.

Exhibit C prices additional mandatory-optional items beyond this table: iQIES ingestion (CMS-1539), PAVE and DHCS PED data interfaces, BreEZe checks, DOJ records search, CABS/OASYS feed, mobile user portal, role-based template/content editing, and (placeholder) data migration/conversion/cleansing. The State chooses which MO items to exercise.

5. Rollout — Three Phases by Facility Group

Implementation adds facility and application types to the Power Platform in three go-lives, each with its own training, operational readiness, stabilization, and production support:

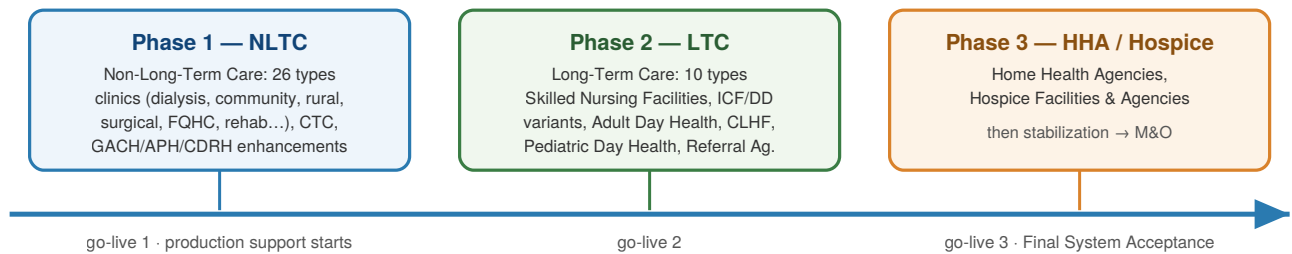


Figure 3 — Phased rollout. Each phase includes UAT, training, go-live, and stabilization; production support also covers the pre-existing GACH-APH build from Phase 1 go-live onward.

License renewal applications (Provider Licensing Unit) and the Provider Certification Unit's workload are additional in-scope functionality whose rollout sequencing will be discussed during the project. Training spans ~100 CAB staff, ~15,000 providers, 16 district office employees, and ~20 Fiscal staff across the three phases.

6. Why the Enhancement Is Necessary

The business case is explicit throughout the RFP:

- **Statutory compliance.** Review timeframes are mandated in the Health & Safety Code (100 days for most clinic/hospital types, 30 days for affiliate clinics and ICF/DD, 120 days proposed for SNFs under AB 1502, <120 days for renewals). With a 231-day average age for open applications, CDPH cannot reliably meet mandates with manual processing.
- **Error prevention at the source.** The single largest cause of delay is incomplete submissions (72% overall). The GACH/APH online experience already proved that guided electronic forms with validation produce more complete, more correct applications. Extending this to the other 33 types attacks the root cause.
- **Eliminating duplicate work.** Analysts re-key online and paper data into ELMS and manually query a dozen verification systems. Integration removes re-keying (and the duplicate-record and typo problems it causes) and automates checks.
- **Payment modernization.** Paper checks cause bounced payments, cash-flow unpredictability for providers, and untraceable refunds. Electronic payment through First Data fixes intake-to-fiscal handoffs.
- **Patient access.** Delayed licenses mean delayed beds and services. Faster, cleaner licensing directly affects healthcare capacity.
- **Workload growth without headcount.** CDPH's strategic plan calls for technology-driven efficiency to absorb growing workload (11,000–14,000 applications/year plus 15,000+ renewals) without additional positions.
- **Enterprise strategy.** CAB Online is a cornerstone of the FoPH Initiative 2 "one-stop" licensing/surveillance backbone and standardizes on CDPH's chosen Microsoft ecosystem.

7. How the Project Will Run (Contract Mechanics)

7.1 Term and timeline

Item	Detail
Base term	38 months = 26 months DD&I (including 2 months stabilization) + 12 months M&O year 1 (concurrent with warranty).
Options	Up to four 1-year M&O extensions (knowledge transfer, production support, minor enhancements) — max 86 months / ~7 years 2 months.
Key dates (Addendum 5)	Proposals due 3/4/2026 noon PT; evaluation 3/5–4/2/2026; negotiations from 4/3/2026; BAFO 5/12/2026; award/contract execution ~8/17/2026.
Payment	Fixed-price, deliverables-based; M&O paid monthly in arrears; 15% withhold released at Final System Acceptance; unanticipated tasks capped at 10% of DD&I cost.

7.2 Methodology and deliverables (Exhibit D)

Delivery follows an **Agile Scrum** framework (product backlog, sprint backlogs, daily scrum with State participation, sprint reviews/retrospectives, releasable increments), wrapped in state-required deliverables across six categories: **(1) Project Planning** (kickoff within 10 days; schedule, product roadmap, PMP, deliverable/OCM/communication, technology-recovery, disaster-recovery, knowledge-transfer plans within 45 days), **(2) Design** (validated RTM, architecture design specs, database design specs, integration & interface specs, implementation plan + BIA), **(3) Development** (system development plan, user stories per release, configure/build with sprint artifacts, defect resolution plan, system & phase test plans, testing incl. UAT support, system management plan), **(4) Implementation** (the CAB Online system per phase, operational readiness, training plan and end-user training, release management plan, go-lives, help desk plan/services, updated as-built docs, knowledge transfer, manuals, production support plan, stabilization, Final System Acceptance), **(5) Post-Implementation** (closeout, M&O plan and services), and **(6) OCM** (organizational change management materials and activities).

7.3 Team, support, and service levels

16 named Key Staff roles are mandatory (Engagement Manager, Project Manager, Scrum Master, Solution Architect, System Configuration & Integration Lead, Lead BA, Lead Developer, Lead DBA, QA Testing Lead, Release Management Lead, Lead Security Analyst, Solution Operations Lead, OCM Lead, Communications Lead, Developer, QA Tester), each with detailed mandatory experience minimums (e.g., Solution Architect: 3+ years Cloud/Power Platform experience; PM: PMP/PgMP; Security: CISSP/CISM/Security+/CISA; OCM: ADKAR/Prosci). All work must be performed within the continental U.S. (no offshore), primarily remote, with FIDO-compliant devices (YubiKey standard) for CDPH access.

Help desk: uses Dynamics 365's help-desk feature; ITIL three-tier model (L1 ≤1 business day, L2 ≤2, L3 ≤4, with P1–P4 priorities); contractor provides non-technical support for 1 year post-go-live and technical support until State resources are trained/available. **SLAs** carry liquidated damages — headline SLA is 99.9% production availability 24×7; key-staff replacement delays are charged at 8 hours/day of that role's rate.

7.4 What bidders must submit and how they are scored

Volume 1: administrative items (cover letter, confidentiality, GenAI disclosure, DVBE $\geq 3\%$ participation, etc.), qualifications (bidder qualification/reference forms; 16 key-staff qualification/reference forms), Exhibit B requirement confirmations (every mandatory requirement answered "Yes"), Exhibit D deliverable confirmations, and narrative responses on approach. Volume 2: the Exhibit C cost workbook, sealed separately. Scoring: **cost is 50%** of total points (lowest total evaluated cost favored); the rest comes from scored qualifications, desirable experience, and narratives; small-business preference (5%), DVBE incentive, and TACPA can adjust ranking. Mandatory misses disqualify.

8. Key Takeaways for the Proposal

- **This is a platform-constrained configuration/extension project, not a technology-selection project.** The State has already chosen Microsoft Dynamics 365 / Power Platform / Azure inside CDPH tenants, and the GACH-APH baseline will already be live on Power Platform before contract start. Bids proposing alternative platforms are non-responsive to the environment.
- **Position the solution as "extend and integrate," not "replace."** ELMS remains the system of record; the contract's core engineering value is (a) 33 new facility-type application experiences with validation, (b) the ELMS bidirectional integration that kills re-keying, (c) First Data payment integration, and (d) a dozen verification interfaces (mostly MO — price them carefully in Worksheet 5).
- **Respect the phasing:** NLTC → LTC → HHA/Hospice, with production support obligations starting at Phase 1 go-live and covering the pre-existing GACH-APH build too — including an assumed ~20% backlog overhead from the legacy systems.
- **Cost discipline matters more than anywhere:** 50% cost weighting, fixed price, 15% withhold, 10% unanticipated-task cap, liquidated damages tied to SLAs and key-staff replacement.
- **Staffing is a compliance gate:** 16 key staff with hard experience minimums and certifications; references validated; only the PM role cannot be shared; no offshore work.
- **Evidence of Power Platform delivery at scale in public sector** (and healthcare licensing/permitting domain knowledge) will map directly onto the mandatory and desirable qualification scoring.

Glossary quick reference: CAB — Centralized Applications Branch · CHCQ — Center for Health Care Quality · L&C — Licensing & Certification Division · GACH/APH — General Acute Care / Acute Psychiatric Hospital · ELMS — Electronic Licensing Management System (2005, system of record) · AEM — Adobe Experience Manager · FoPH — Future of Public Health initiative · CHOW — Change of Ownership · NLTC/LTC — (Non-)Long-Term Care · HHA — Home Health Agency · PAVE — DHCS Provider Application and Validation for Enrollment · CABS/OASYS — Caregiver Applicant Background System / Online Application System · iQIES — Internet Quality Improvement and Evaluation System · DD&I — Design, Development & Implementation · M&O — Maintenance & Operations · RTM — Requirements Traceability Matrix · OCM — Organizational Change Management.

Sources: RFP 26-10053 A5 Part 1 — Bidder Instructions (2/25/2026); Exhibit A — Statement of Work; Exhibit B — Functional and Non-Functional Requirements workbook; Exhibit C — Cost Worksheets; Exhibit D — Deliverables Table; CDPH "Apply for Licensure — Electronic" program page (eforms.cdph.ca.gov portal description).